BurnScope Assessment Report

Case ID: 0f78a04b-test

Case opened: 2026-09-14 | Report generated: 2026-09-20 00:58

CLINICAL REVIEW ADVISED

Key findings

Severity	2nd degree (partial thickness)
Confidence	83% - model was unsure (low reliability)
Wound area (share of photo)	moderate (27%)
Wound regions	multiple separate regions
Image quality	acceptable

Reason for this recommendation

The image classifier was not confident enough to commit to a severity (best guess: 2nd degree (partial thickness) (confidence 83%)), so the conservative option was taken; more than one separate burned region was detected; the follow-up shows a worsening trend.

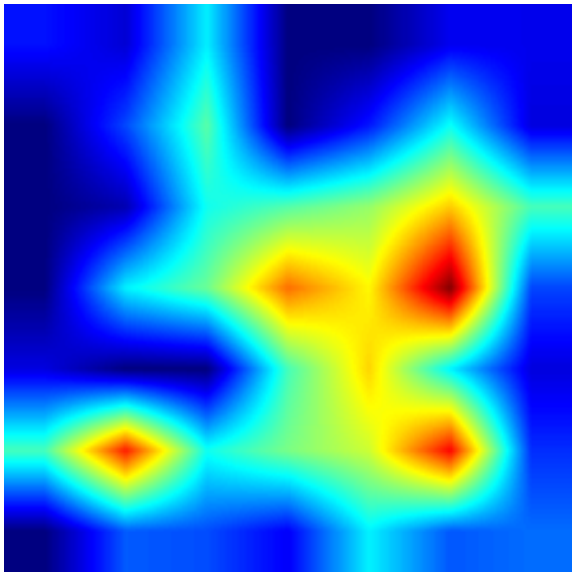
#	Recommended action	When
1	Arrange a clinician review of the burn.	**Within 24-48 hours**
2	Keep the wound clean and covered as described in the guidance below.	**Ongoing**
3	Seek care sooner if pain worsens, redness spreads, there is pus, a bad smell or fever.	**Any time**
4	Re-photograph the wound for the follow-up check.	**On 2026-09-22**

Change since the first visit

	Baseline visit	Current visit	Change
Severity	1st degree (superficial)	2nd degree (partial thickness)	**worse**
Wound area	12%	27%	**+15 points**

The burn is larger and deeper than at the first visit.

What the model looked at



Grad-CAM overlay: warmer colours mark the image regions that most influenced the severity prediction. It shows where the model looked, not a diagnosis.

Guidance summary

Burns in children

Children have thinner skin, which burns faster, deeper and at lower temperatures than an adult's skin. Every infant or child with a burn should be medically assessed.
Call an ambulance (emergency number) if the burn is severe, involves the face, hands, genitals, airway (nose, mouth or throat) or eyes, or is larger than the child's forearm, if the child has breathed in smoke, or if the burn was caused by chemicals or electricity.
See a health professional urgently if the burn is about the size of a 20-cent coin or larger, is deep and looks white (even if it is not painful), is blistering, causes severe pain or pain that continues despite pain relief, or if the child was too upset to be cooled for 20 minutes. A burn that goes all the way around a limb, or any burn in a baby under 12 months, also needs urgent medical review. **[1][2][3][4][5]**

Children have thinner skin, which burns faster, deeper and at lower temperatures than an adult's skin. Hold the burned area under cool running water continuously for 20 minutes, starting as soon as possible.
If the burn is large, do not cool it for more than 20 minutes, because children can become dangerously cold (hypothermia) quickly; cooling can be paused if the child becomes cold. Use cool, not cold, water, and avoid a full cold bath. Keep the rest of the child warm with a blanket or towels while cooling.
If the child is too upset to be cooled, or cooling cannot be continued, get medical help. **[1][4][6]**

Warning signs and infection

Toxic shock syndrome is a rare but serious complication that can follow even a minor burn, and it can make a child very unwell quickly. In the days after a burn, seek emergency medical help at once if a child has: a fever of 38 degrees C or higher, vomiting or diarrhoea, a widespread red or sunburn-like rash, red lips or tongue or red sore eyes, drowsiness, confusion or not seeming themselves, eating or drinking much less than usual, fewer wet nappies, dizziness or fainting, difficulty breathing, or blotchy purple (mottled) skin. **[7][8][9]**

Get medical advice promptly if a burn shows any of these signs of infection: pain that is getting worse or is not explained by the size of the burn, redness or swelling that is spreading, pus, a bad smell, or a fever of 38 degrees C (100.4 degrees F) or higher. A dressing that becomes wet, stained, smelly or loose needs to be changed by a health professional. Do not burst blisters. **[3][9][10][11]**

First aid and emergency care

Cool the burn as soon as possible with cool running water for 20 minutes. Cooling is still worthwhile if it is started up to 3 hours after the burn, but the sooner the better. Use cool tap water, not ice-cold water. If there is no running water, cool bottled water or a wet towel can be used.
Do not use ice or ice water. Do not put creams, lotions, oils, butter, toothpaste or other home remedies on the burn, and do not break blisters. Remove clothing and jewellery near the burn, but do not pull off anything that is stuck to the skin.
After cooling, cover the burn loosely with a clean, non-stick dressing or plastic cling film. Lay cling film over the burn rather than wrapping it around a limb, and do not use cling film on the face or on chemical burns. Keep the person warm. **[2][3][4][12]**

Wound care and dressings

All burn areas are cleaned and loose dead skin is removed. Blisters are de-roofed, collected plasma removed and an appropriate dressing is applied. Biological dressing is preferred dressing material if available. Collagen is cheap and good skin substitute for partial thickness burns. If this is not available then a non-adhesive dressing in the form of paraffin gauze impregnated with 1% [medication], colloidal silver, ionic silver cream or [medication] cream is applied. Dressing is further supported with absorbent cotton pads (Gamgee). **[13]**

Healing of Burn Wound:
A 2nd degree superficial or superficial partial thickness burn heals by epithelialisation and takes 10 to 20 days to heal
A 2nd degree deep or deep partial thickness burn heals by both wound contraction and epithelialisation
A 3rd degree or full-thickness burn heals only by contraction after eschar separation **[14]**

For full thickness burns:
All above mentioned products can be applied as a temporary cover for full thickness burns after excisional therapy has been done & skin grafting cannot be done because of limited or non availability of donor site.
Best dressing is patient’s own skin graft (autograft). Therefore the role of excisional surgery & grafting is of great consequence in deep burns.
Sometimes when donor site is inadequate; especially in children, skin graft from parent/sibling (homograft) can be taken to provide temporary cover in order to reduce the septic load & promote the onset of anabolic phase of healing. But, homograft has to be always replaced with an autograft in due course of time.
Excision surgery & skin grafting in burns
All full thickness burns require excision of dead skin. Douglas Jackson of Birmingham hospital of U.K. was probably amongst initial few people who suggested surgical intervention in burn patients. Zora Janzekovic of Yugoslavia was amongst the first who suggested the concept of tangential excision of burns which was initially implemented for deep dermal burns.
Gradually, same theory was extended for full thickness burn also and it was called as sequential excision. **[15]**

Medication names are deliberately left out of this report; ask a clinician which treatments are appropriate.

References

[1] KidsHealth NZ — First Aid for Burns & Scalds in Children (reviewed 7 October 2024)

[2] Health NZ — Burns first aid (July 2026)

[3] Royal Children's Hospital Melbourne — Burns and scalds: prevention and first aid

[4] ANZCOR — Guideline 9.1.3: First Aid for Burns (March 2023)

[5] NHS — Burns and scalds

[6] Royal Children's Hospital Melbourne — Burns: acute management, clinical guideline (April 2026)

[7] Chelsea and Westminster Hospital NHS Foundation Trust — Toxic shock syndrome (TSS) after a burn, patient leaflet

[8] Alder Hey Children's NHS Foundation Trust — Burns and scalds, patient information leaflet

[9] NHS Scotland, Yorkhill Emergency Department — Advice on looking after your child's burn (reviewed February 2015)

[10] Mass General Brigham — Burn wound check (child)

[11] NPPMBI — Chapter 7: Local Wound Management (Government of India)

[12] NHS — Burns and scalds: treatment

[13] NPPMBI.pdf — Chapter 7: Local Wound Management — pp.24-26

[14] Indian STG — Chapter 4: Burn Wound Management (Section F) — pp.33-36

[15] NPPMBI — Chapter 7: Local Wound Management — pp.24-26

Nearby care options

Facility	Address	Distance
City Burns Unit	12 Example Road	3.2 km

Follow-up

Next check-in	2026-09-22	Re-photograph the wound and run the check again.

There is no automatic reminder; please set your own.

This is a machine-generated assessment that supports, and does not replace, clinical judgment. A qualified clinician should confirm severity and management in person. Shared with the patient's consent to: dr.test@hospital.org.